Perry Spine & Injury Center

6149 Rose Inlet
Houston, TX
Phone 281-555-6900

Itemized Medical Bill

Patient:	Phillip Marshall
Date of birth:	05/17/1994
Insurance claim:	C-1044
Date of injury:	02/14/2024
First visit:	02/20/2024
Last visit:	07/04/2024
Number of visits:	20

Services

Service	Units	Charge
99203 New patient evaluation	1	$276.53
98941 Spinal manipulation	5	$641.48
97110 Therapeutic exercise	4	$715.09
97124 Massage therapy	3	$258.09
72040 X-ray, cervical spine	2	$1,594.37
20552 Trigger point injection	5	$2,544.88
TOTAL		$6,030.44

Recent visits

Date	Service	Charge
05/27/2024	97110 Therapeutic exercise	$174.58
06/05/2024	97110 Therapeutic exercise	$159.89
06/12/2024	98941 Spinal manipulation	$135.69
06/22/2024	97110 Therapeutic exercise	$171.26
06/27/2024	97110 Therapeutic exercise	$209.36
07/04/2024	98941 Spinal manipulation	$129.52

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Treatment History and Summary

Patient:	Phillip Marshall
Treating provider:	Thomas Hall, DC
Diagnosis:	concussion

Treatment history by month

Month and services	Visits	Charges
February 2024: evaluation, imaging	2	$850.55
March 2024: massage therapy, pain management	4	$1,312.93
April 2024: chiropractic adjustment, imaging, massage therapy, pain management	4	$1,660.04
May 2024: chiropractic adjustment, pain management, physical therapy	5	$1,401.20
June 2024: chiropractic adjustment, physical therapy	4	$676.20
July 2024: chiropractic adjustment	1	$129.52

Treatment summary

Clinical Treatment Summary

The patient presented following a motor vehicle collision in which the patient's vehicle was struck while turning left. The reported mechanism, a direct impact with head movement during the turn, is consistent with the concussive injury diagnosed at intake. Over the course of care, the patient attended 20 visits, a somewhat longer course than is typical for an injruy of this type, with symptom management and graded activity adjusted as the patient progressed.

Examination findings support the diagnosis of concussion, including documented balance complaints, light sensitivity, and reduced performance on certain cognitive screening items. Other neurological tests, including cranial nerve assessment, strength, and reflexes, returned normal results. Imaging was not indicated based on the clinical presentation.

The patient's medical history reveals no prior injury to the head or neck region. Billing records for the episode of care reflect each service once, with no duplicated or repeated charges.

Current status: symptoms have improved but not fully resolved. Ongoing follow-up, graded return to activity, and symptom monitoring are recommended.